

Capacity & Scheduling Vendor Demos: Comparison

Arya (Sep 28), Axle Health (Sep 22), CareStitch (Sep 23), VitalisCare (Sep 24). Vendors in alphabetical order. Sources: demo calls and each vendor's RFP return.

Compassus Home Health
Internal, for discussion

What each product is

- Arya**: an agent that texts the patient, ranks clinicians with a suggested visit time, and writes back to HCHB after office approval. Clinicians use text and short links, no app. HCHB by Citrix screen automation.
- Axle Health**: a home health scheduling product that recommends clinician, day and time with the factors shown and plots the episode in visit windows. HCHB write-back built but switched off pending faster HCHB data.
- CareStitch**: a scheduler workspace with capacity by clinician and day, a staffing-request workflow and care-team chat. It filters eligibility; the scheduler decides. HCHB integration is a read-only beta.
- VitalisCare**: a scheduling product with a live HCHB integration, used today in hospice. It ranks providers and suggests better visit days. Authorization, episode/LUPA logic, therapy and patient messaging would need to be built.

Variable coverage on the calls (79 variables)

	Capacity (22)	Scheduling (45)	Engagement (12)	Total (79)
Arya	91% / 41%	69% / 36%	92% / 50%	78% / 39%
Axle Health	86% / 73%	76% / 51%	83% / 25%	80% / 53%
CareStitch	95% / 73%	62% / 36%	83% / 33%	75% / 46%
VitalisCare	91% / 55%	62% / 38%	92% / 0%	75% / 37%

Discussed on the call (includes roadmap) / shown in the software. MVP variables shown (49): Arya 43%, Axle 59%, CareStitch 49%, VitalisCare 41%. Call time was spent differently: Arya's demo skipped the capacity view its RFP return describes, so its "shown" capacity figure is low.

Not covered by any vendor (7): add-on orders, acuity to skill level, cognitive/dementia constraints, missed-visit documentation (all MVP), lunch/documentation pattern, clinician safety, infection-control sequencing.

Open items for the on-site

Arya	Sandbox load test of HCHB reads and writes at our volume (15-20 min on the call vs under 5 min in the return). The RFP capacity check shown on real data. Approval kept on per action; a view and sign-off for learned rules. HCHB customer count; whether Aveanna's +11% was home health; LUPA and recert.
Axle Health	A data plan: intake feed for new patients, HCHB refresh target, a date to turn write-back on. Our scenario end to end with LUPA and a referral decision. Source behind the RFP impact numbers; partnership terms; company size.
CareStitch	A dated HCHB production plan for read and write. The automation work they mentioned and a date for recommendations. A date for patient text confirmation. A 60-day episode run. References in writing.
VitalisCare	Written scope, dates and cost for the home health build (auth, episode/LUPA, therapy, patient messaging). The HCHB refresh at Compassus (3 h on the call vs 10-30 min in the return). Call-Off Center shown live. A home health reference.

RFP scorecard, and what the demos add

Section (weight)	Arya	Axle Health	CareStitch	VitalisCare
HCHB (20)	16 Live, small base. = Citrix RPA confirmed; 15-20 min on the call, under 5 in the return	0 None, no path. ▲ to 2 possible: our intake feed offered as a path; write-back still off	2 Roadmap, no date. = not covered on the call	16 Live, small base. ▼ possible: 3-h refresh on the call vs 10-30 min in the return; live base is hospice
Capacity (12)	11/12. ? unverified : referral-level check in the return, not shown	11/12. = capacity by zip shown	11/12. = open points by address shown; no branch total	8/12. = branch roll-up still roadmap
Scheduling (12)	12/12. = matching, route, call-out shown	12/12. = episode plotting and call-out flow shown	8/12. = manual assignment confirmed	9/12. = ranking and better-day flags shown
Engagement (12)	10/12. ▲ +1 possible : patient texts shown live (return said one customer)	10/12. = arrival-window text shown	7/12. = confirmation still roadmap	4/12. = no patient messaging
Sophistication (20)	4 Runs it. = outreach and call-outs ran with no per-event approval; new-case staffing gated for the demo	3 Recommends. = recommendations with factors shown	2 Checks it. = automation still in research	3 Recommends. = scheduler approves ranked options
Clinician fit (12)	2 Workable. ▲ to 3 possible : preferences, text-first, clinician's yes shown	3 Good. = clinician finalizes own day	3 Good. = field-built clinician tools shown	2 Workable. ? to discuss : all-day location and route-adherence review shown
Partnership (12)	3. =	2. =	3. =	3. =
Total · band	84 Advance · 88 if both ▲ apply	63 Conditional-Hold · 65 if ▲ applies	56 Conditional-Hold · no change	67 Consider · 63 if ▼ applies

Scores are the formal RFP scorecard (9.11.26). Arrows mark where what we saw on the call differs from the written return, using the scorecard's own rubric: ▲ the demo supports a higher mark, ▼ a lower one, ? not verified on the call, = the demo matched the return. The formal scorecard is unchanged until the team re-scores. Scorecard notes to check: Axle's uptime note (99.5%) differs from its return (99.9% SLA, under 1 h down).

Key differences (call and RFP return)

	Arya	Axle Health	CareStitch	VitalisCare
HCHB integration	Citrix screen automation, read and write, live since mid-2025 (RFP). Reads 15-20 min on the call; under 5 min in the RFP. No HCHB customer count.	Built, switched off. RFP: log ship up to 30 min stale; waits on HCHB's data move. Intake feed offered for new patients.	Read-only screen beta, never in production; no write-back. RFP: FHIR/HL7 preferred, timing depends on HCHB.	Live: log ship reads, RPA writes. About 3 h at Compassus on the call; 10-30 min in the RFP.
Capacity	RFP: referral-level check (SOC-eligible staff with room, episode impact), clinician capacity rolled up by branch. Not shown.	Shown: capacity by zip. RFP: every clinician who could take the referral, with forward capacity.	Shown: open points by clinician and day for an address. No branch total, by design (RFP).	Not shown. RFP: room by discipline and clinician; branch view being built.
Home health logic	Capacity windows (SOC 24 h, Medicare week, fixed day); visits held until auth clears. No LUPA, recert or OASIS in call or RFP.	Visit windows, Medicare week, front-loading, SOC/eval/recert checks. No LUPA in call or RFP.	Relies on HCHB. RFP: no orders, auth or windows ingested or enforced.	Hospice rules today. Auth, episode, LUPA and therapy to be built.
Patient contact	Two-way text live (discharge check, preferences, arrival time). Voice and Spanish claimed.	Arrival-window text live. RFP: voice, text, email, with a person in the loop advised.	Click-to-call with a call log. Text confirmation on roadmap.	None today, no date (RFP).
Customers (RFP)	~60 orgs, "even split" HH, hospice, PDN, home care; 3 largest >30k ADC.	~25 orgs, 80% home health; largest 11k ADC.	79 orgs, 69% home health; largest 5.4k (one branch).	6 orgs, 100% hospice in production; largest ~4k ADC.

Coverage method: each Variable Inventory row marked shown, discussed or not covered for each call. "RFP" marks a claim from the vendor's questionnaire return that was not seen in a demo. This page compares; it does not rank or recommend.